

Referring Physician:

UID: 8DFDB3AE-8458-4F8C-96D8-A022E75D28C9
TCGA-FY-A40L-01A-PR

Redacted



DOB: Age: Gender

Ref#: Hosp#: Provider Group:

Date of Service: Date Received: Outpatient Case #:
Room: Bed: Date Reported:

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is [circle]:	QUALIFIED	DISQUALIFIED
Reviewer Initials		
Date Reviewed		

A. LEFT PARATRACHEAL TISSUE, BIOPSY:

- METASTATIC PAPILLARY THYROID CARCINOMA IN LYMPH NODE.
- The metastatic focus measures 0.2 cm in diameter, and comprises approximately 10% of the lymph node tissue volume.

B. CENTRAL LYMPH NODE, BIOPSY:

- METASTATIC PAPILLARY THYROID CARCINOMA IN LYMPH NODE.
- The metastatic focus measures approximately 0.5 cm in diameter and comprises approximately 90% of the lymph node tissue volume.

C. THYROID, THYROIDECTOMY:

- MULTIFOCAL PAPILLARY THYROID CARCINOMA, INVOLVING LEFT AND RIGHT LOBES.
- The largest tumor nodule (inferior aspect of right lobe) measures 1.7 cm in diameter.
- Tumor extends to inked and cauterized surfaces of the resected thyroid gland (at the inferior aspects of left and right lobes).
- PARATHYROID ADENOMA, AT LEAST 0.9 CM IN DIAMETER, ADJACENT TO INFERIOR POLE OF RIGHT THYROID LOBE.
- CHRONIC LYMPHOCYTIC THYROIDITIS.
- SMALL BENIGN LYMPH NODE ADJACENT TO THYROID.

COMMENT: Three tumor nodules are grossly identified, the largest of which is identified in the lower pole of the right thyroid lobe (1.7 cm in diameter). Two additional tumor nodules are identified in the left lobe, a 0.4 cm nodule in the upper portion, and a 1.1 cm more inferiorly. Carcinoma extends to the cauterized and inked margins in the inferior portions of both left and right thyroid lobes.

ICD-3 Carcinoma, papillary, thyroid
Site: thyroid, NOS C73.9
8260/3

Tumor Staging Information

Data derived from current specimen. Staging in accordance with or modified from AJCC Cancer Staging Handbook, Ed, and CAP protocol.

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Procedure:	Thyroidectomy.
Specimen type:	Total thyroidectomy, with separately submitted lymph node biopsies.
Specimen integrity:	Intact thyroid.
Specimen size:	See gross description.
Tumor focality:	Multifocal papillary thyroid carcinoma.
Tumor laterality:	Bilateral.
Histologic type:	Papillary thyroid carcinoma.
Lymphovascular invasion:	Not identified, but multiple positive lymph nodes are identified.
Tumor capsule:	Not applicable. Tumor grossly noted to invade perithyroid tissue at the time of surgery.
Extrathyroid extension:	Extrathyroid extension identified by surgeon at the time of surgery; tumor extends to inked cauterized edge of thyroidectomy specimen.
 LYMPH NODES:	 Metastatic papillary thyroid carcinoma identified in two of three lymph nodes.
 MARGIN EVALUATION:	
Distance to closest margin:	Carcinoma extends to inked cauterized thyroidectomy surface.
 PATHOLOGIC TUMOR STAGING:	
Primary tumor:	pT4a (moderately advanced disease. Intraoperatively, tumor noted to invade outside of thyroid).
Regional lymph nodes:	pN1b.
Number of nodes examined:	3.
Number of nodes involved:	2.
Distant metastasis:	Not applicable.
Pathologic stage:	IVA.
 Additional pathologic findings:	 Parathyroid adenoma.
 Comment:	 The tumor stage (pT4a) as reported above is based in part on extension of tumor to the cauterized inked thyroid tissue surface, and in part on the intraoperative finding of tumor invading outside the thyroid into the region of the recurrent laryngeal nerve. The pathology findings were discussed with Dr. _____ on _____.

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Signed by

Source of Specimen:

- A. Left tracheal tissue
- B. Lymph node; Central node
- C. Thyroid; Total thyroid

Clinical History/Operative Dx:

Thyroid cancer

Intraoperative Diagnosis:

- A. Left paratracheal tissue: Lymphoid tissue. (Dr. . The intraoperative interpretation(s) was/were performed and rendered at .

Gross Description:

A. The specimen is labeled left paratracheal tissue and is received without fixative. It consists of a 0.2 x 0.2 x 0.2 cm fragment of yellow-tan tissue which weighs 0.0263 grams. It is entirely submitted for frozen section as FSA. The tissue remaining from frozen section is submitted for permanent section in cassette A1. (

B. The specimen is labeled central node and is received in formalin. It consists of two fragments of fibrofatty appearing tissue. The larger fragment measures 1.5 x 1.2 x 0.4 cm and the smaller fragment is firm and pale tan, 0.6 cm in maximum dimension. The larger fragment appears to consist of fibrofatty tissue. It is inked blue. Both specimens are serially sectioned and all tissue is submitted in cassette B1.

C. The specimen is labeled total thyroid and is received without fixative. It consists of a total thyroidectomy specimen weighing 10 grams. A long black silk stitch marks the right lobe of the thyroid and a white medium stitch marks an area of possible parathyroid adenoma according to the surgeon. The right thyroid lobe measures 3.3 cm from superior to inferior, 2.5 cm from medial to lateral, and 0.8 cm from anterior to posterior. The left thyroid lobe measures 3.1 cm from superior to inferior, 2.4 cm from medial to lateral, and 1.1 cm from anterior to posterior. There is very little if any recognizable isthmic tissue. The anterior surface of the right thyroid lobe is inked blue, the anterior surface of the left thyroid lobe is inked green and the posterior surfaces are inked black. There is no grossly identifiable parathyroid tissue. Serial sections of the right thyroid lobe reveal an inferior ill-defined area of firm tan-white discoloration which is

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1.5 cm in maximum dimension. Sections of the left thyroid lobe reveal a 0.4 cm focus of firm tan-white tissue in the superior pole, as well as a relatively well-circumscribed, tan-white mass in the central inferior portion of the left thyroid lobe measuring 1.1 cm in maximum dimension. Representative sections of the left inferior thyroid mass are submitted for research purposes. The specimen is entirely submitted. Section summary: C1-C4) right thyroid lobe from superior to inferior, C5) superior left thyroid lobe with small nodule, C6-C9) left thyroid lobe from superior to inferior.

Microscopic Description:

A. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

B. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

C. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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